Cash Transfers, Health, and the Transition to Adulthood

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Cash transfers are a popular tool to encourage education

- Low-income countries have successfully used cash transfers to increase levels of education (*Progresa*, *Bolsa Familia*)...
- ...but cash transfers in high-income countries often have different effects than in LICs
- Also in high-income countries approaches like this have been proposed to combat inactivity (Basic income or participation income as in Atkinson (1996))

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- ...but cash transfers in high-income countries often have different effects than in LICs
- Also in high-income countries approaches like this have been proposed to combat inactivity (Basic income or participation income as in Atkinson (1996))
- An especially striking problem in high-income countries is the amount of young individuals with (mental) health problems that end up inactive in the labor market
- My setting: adolescents with health problems in Finland

Figures

In this paper...

- I explore the effects of Youth Rehabilitation Allowance, a program that (in 2023) provided 800 euros per month for 16 000 adolescents with health problems
- I estimate the short- and long-run effects of this program using an instrumental variables design
- Despite some intended short-run effects, the benefit has negative effects on labor market success in the long run
- I present a simple model of human capital accumulation that can explain the effects and discuss implications
  - Rather preliminary

What is Youth Rehabilitation Allowance (YRA)?

- Launched in 1999 as an alternative to Disability Insurance for adolescents; currently serves 16 000 recipients (as of 2023)
- **Purpose:** To support individuals with health challenges in pursuing education and employment, reducing reliance on disability benefits in the long-run.
- **Key Details:**
  - Conditional on medical need assessment, rehabilitation plan, and school/rehabilitation attendance
  - Targeted at 16–19-year-olds
  - Can continue for longer if the "rehabilitative activity" is incomplete
  - Substantial benefit (€500–€800/month)
  - Non-means-tested; no restrictions on usage

Map

Public debate about the benefit

Teachers/doctors:

– The money paid without compensation has not promoted the young person's rehabilitation, but has turned against the young person and made him passive, says one teacher, speaking anonymously.

According to teachers, young people have not applied for summer jobs or gone on internships, for example, because they already receive money.

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Students:

Elias Rantasuo, who is studying to be an electrician, is familiar with the youth rehabilitation grant.

– A couple of my friends get it. They use the money for their hobbies, such as cycling and motocross, and just living in general. It's really good that young people can get that kind of money, says Rantasuo.

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Government:

Nuoren kuntoutusraha kohdistuu oikein: nuoriin, jotka tarvitsevat muita enemmän tukea, palveluita ja etuuksia

Julkaistu 18.8.2022

Lapset ja nuoret

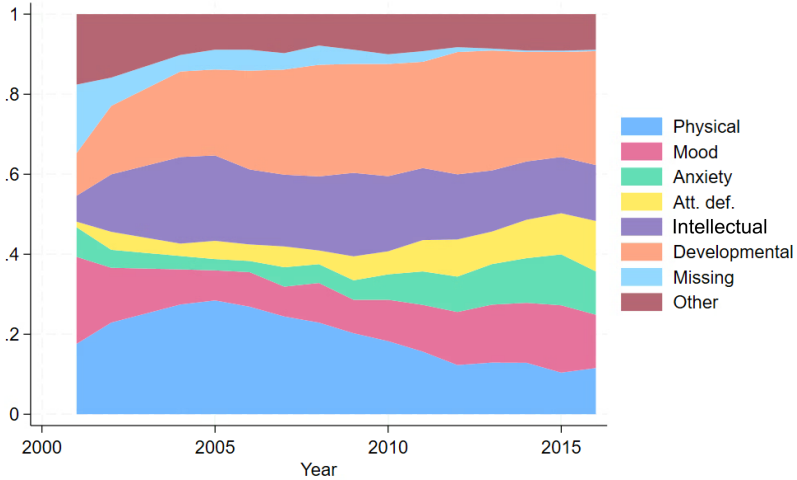
Nuoren kuntoutusraha suututtaa, ministeriö ja Kela puolustavat 800 euron etuutta: "Yhä useampi nuori jatkaa opiskelua ja pyrkii työmarkkinoille"

Kotona asuva 16-vuotias voi saada Kelalta 800 euroa kuussa.

Data

- Applications to the Youth Rehabilitation Allowance (YRA), including acceptance decisions, payments, and examiner identifiers (2004–2017)
- Employment, income, educational attainment, and family status
- Public healthcare use (inpatient/specialized (1995–) and outpatient (2011–)
- Reimbursed prescription drug purchases
- Criminal convictions and enforced debts

Describing recipients: diagnosis codes



Intellectual disability: ICD10 F7*, Developmental: ICD10 F8*, Other: other F-cat.

Summary table

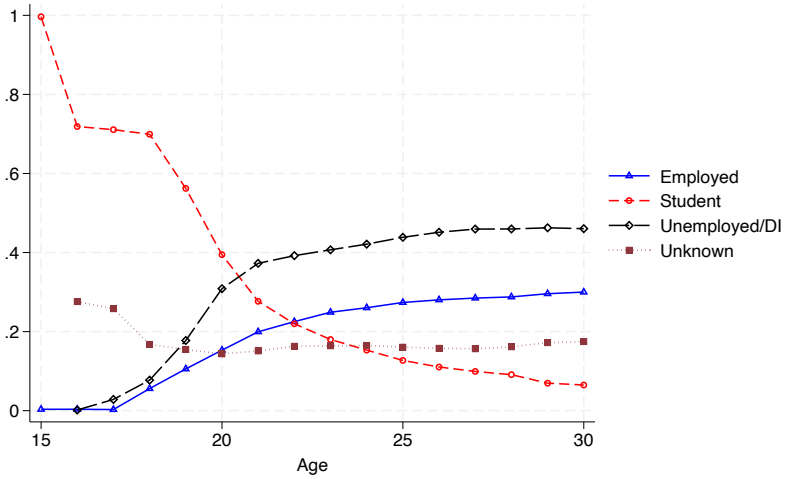
	(1) Gen. pop.	(2) Accepted	(3) Rejected
Male	0.52	0.55	0.50
Student	0.98	1.00	1.00
Employed	0.02	0.00	0.00
Healthcare visits	0.95	8.34	6.51
Anxiety Diagnosis	0.01	0.09	0.08
Depression diagnosis	0.01	0.10	0.08
Other MH diag.	0.03	0.37	0.29
Single parent HH	0.21	0.29	0.30
Urban	0.81	0.78	0.80
Mother age	44	44	44
Father age	46	47	47
Mother DI	0.03	0.08	0.07
Father DI	0.04	0.08	0.08
Mother married	0.70	0.56	0.56
Mother employed	0.80	0.68	0.70
Father employed	0.78	0.67	0.68
Mother taxable income	24912	23961	24470
Father taxable income	34873	32942	33267
Observations	1421282	20830	9741

Notes: all measurements at age 15. Years 2004-2017.

Summary table

	(1) Accepted	(2) Rejected
Ever in YRA	1.00	0.24
Total YRA benefits (€)	25 925	6 027
Reason for application		
Physical	0.18	0.22
Depression	0.12	0.17
Anxiety	0.08	0.11
Developmental	0.27	0.23
Intell. dis.	0.17	0.09
Att. def.	0.08	0.08
Education enrollment (1 year after applic.)		
Basic education/other (=inactive)	0.48	0.33
Vocational high school	0.39	0.50
Academic high school	0.13	0.17
Observations	20830	9741

Describing recipients: labor market status



Notes: *Unknown* includes military service

Identification I

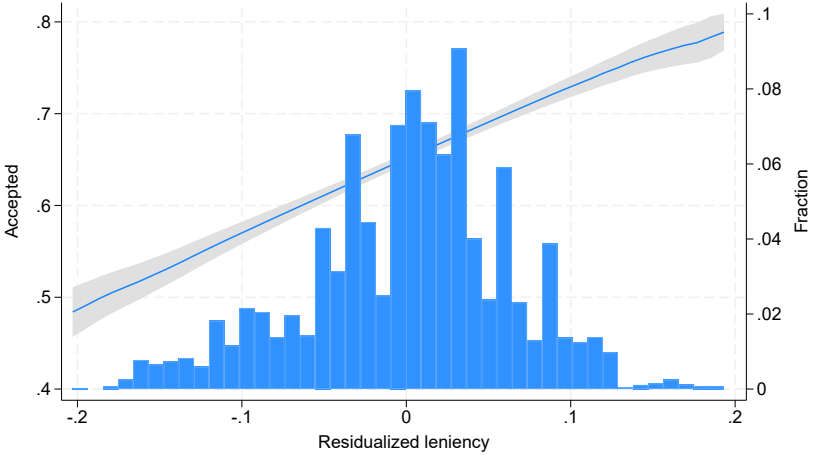
- Applications allocated to examiners from a running queue => essentially random within time × office × language (Source: written communication)
  - First used by (Reiterä, 2021) on rehabilitative activities
  - In total, around 300 judges with on average 80 cases
- I use the (leave-out) acceptance tendency of the examiner as an instrument (Examiner/Judge-IV)
- First stage outcome/treatment: first application accepted
- Assumptions needed: exogeneity, exclusion, relevance, monotonicity, (+extensive margin compliers only (Rose and Shem-Tov, 2024))
- Under these assumptions, identifies the local average treatment effect

Application Formula Choice of endogenous variable Split-sample IV

Identification II

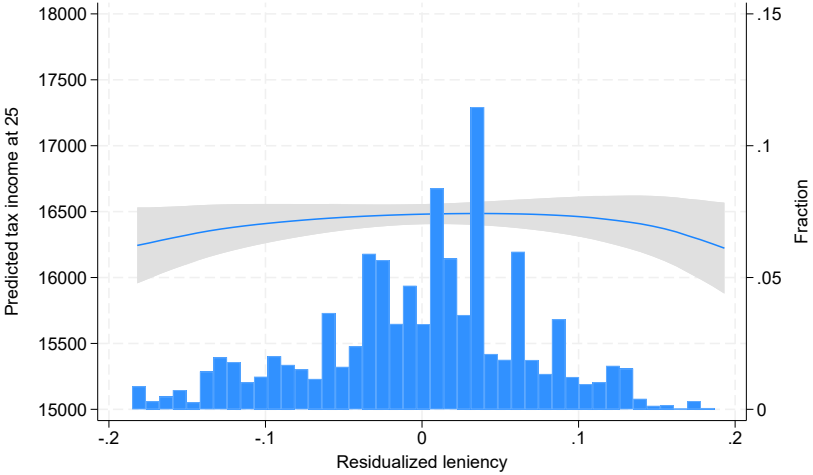
- Report Anderson-Rubin p-values robust to weak instruments (Andrews et al., 2019; Chyn et al., 2024)
- Shrink the estimated instruments using Empirical Bayes (Frandsen et al., 2026) – improves precision
- Key results similar when different choices made in estimation
  - No shrinkage
  - Additional controls + residualization
  - UJIVE (Kolesar, 2013)
  - Binary instruments
  - etc...

First stage

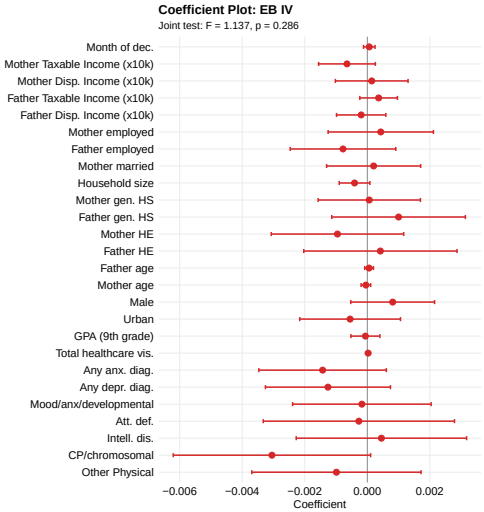


F-stat: 253.4

Balance I


18000
17500
17000
16500
16000
15500
15000
Predicted tax income at 25
.15
.1
.05
0
Fraction
-.2
-.1
0
.1
.2
Residualized leniency

Balance II



With acceptance decision

Conceptual framework

- Through which channel could the YRA affect the recipients' labor market participation?
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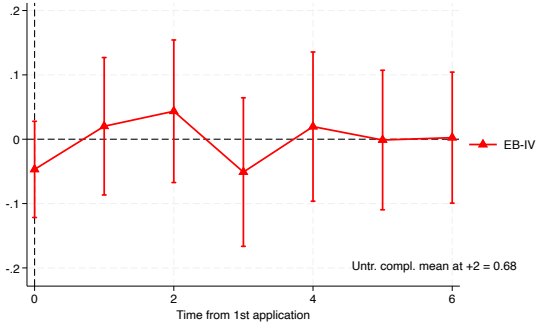
  - - Increased consumption of harmful goods (Evans and Moore, 2011)

Results

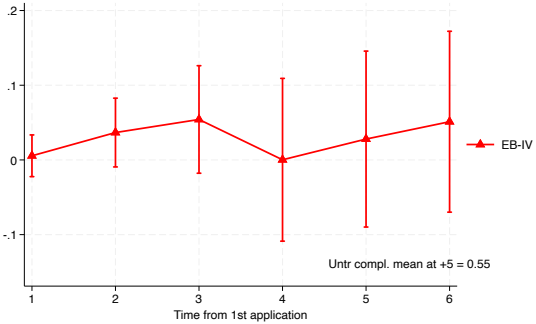
1. Short run results
2. Long run results
3. Heterogeneity and robustness

Short run: modest increase in attendance and graduation

Currently student in secondary education

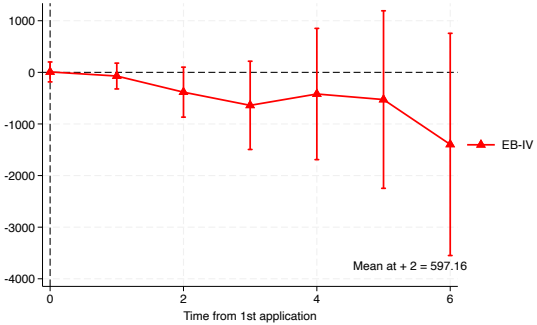


Has secondary education degree

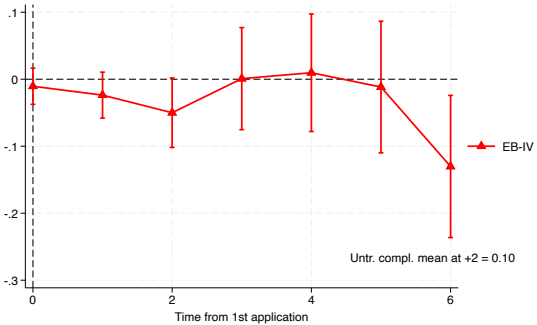


Short run: decreases in working

Wage earnings

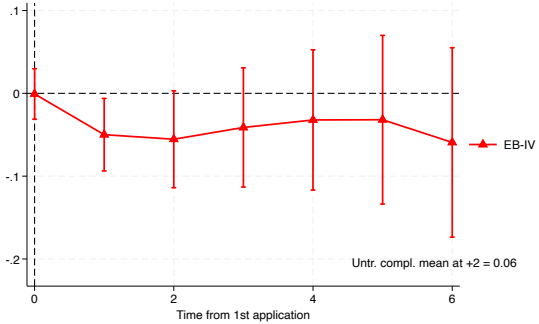


Employed (full time)

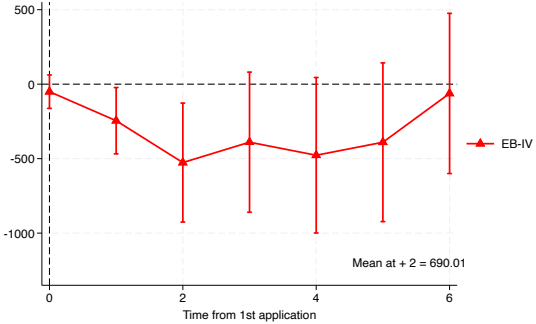


Short run: Would have received other benefits

Disability insurance



Social assistance



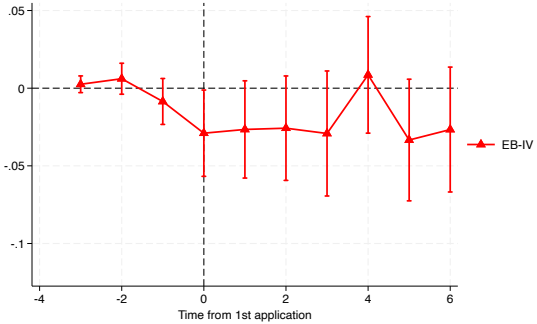
Short run: reduced labor supply

	(1) DI	(2) Employed	(3) Cum. Wage inc. by 20	(4) Student (2ndary)
OLS	-0.120***	-0.061***	-3671.854***	0.036***
	(0.004)	(0.004)	(51.990)	(0.003)
IV	-0.0426	-0.025	-1236.901*	0.017
	(0.0278)	(0.023)	(660.607)	(0.036)
Untreated complier mean	0.084	.111	2916	.607
Sample mean	.32	.0936	2664	.583
AR p-value	0.919	0.271	0.063	0.63
First stage F	391	391	391	391
N	122064	122064	122064	122064
N indiv.	30591	30591	30591	30591

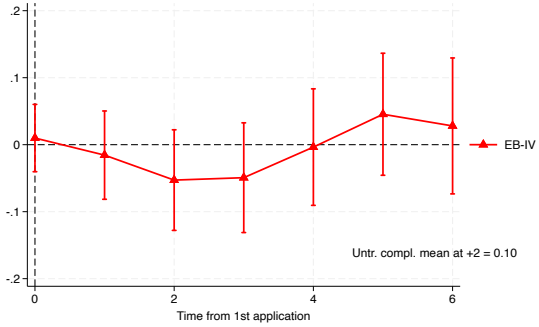
Notes: averages over years 1-5 after application. Figure format

Risky behaviours: Financial distress and addiction

Any addiction diagnosis



In Debt Enforcement



Other short run results

- No clear changes in healthcare visits, drug purchases
- No detectable changes in crime Here
  - Large SEs, but enough to rule out effects from some other papers (Huttunen et al., 2023; Deshpande and Mueller-Smith, 2022; Doyle Jr, 2007)
- Where does the benefit go? Hard to observe but
  - No changes in share living without parents
  - No changes in use of rehabilitative psychotherapy

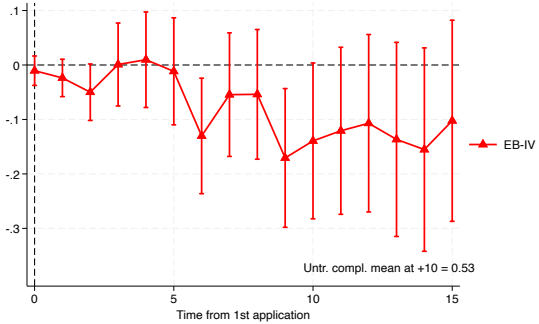
Long run: decrease in employment

	(1) NEET	(2) Employed	(3) Wage inc.	(4) Student
OLS	0.119***	-0.111***	-3743.833***	-0.009**
	(0.007)	(0.007)	(193.139)	(0.004)
IV	0.137**	-0.139**	-2448.250	0.002
	(0.064)	(0.061)	(1750.635)	(0.028)
Untreated complier mean	.428	.452	11720	.105
Sample mean	.586	.317	7845	.0835
AR p-value	0.033	0.023	0.165	0.949
First stage F	243	243	243	274
N	82142	82142	82142	82142
N indiv.	18048	18048	18048	18048

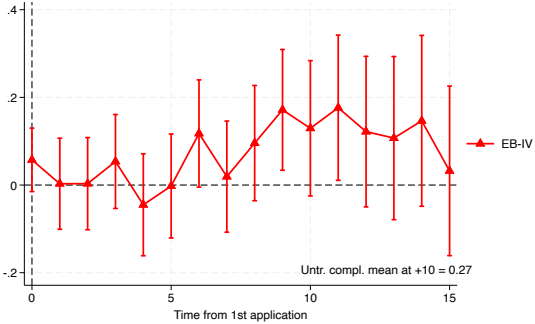
Notes: averages over years 8-15 after application. Figure format

Effects on labor market activity

Employed



NEET



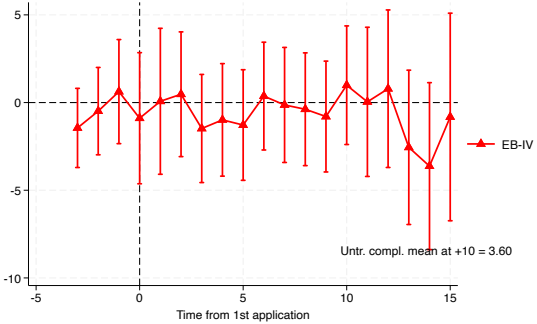
Health/healthcare use

	(1) Hosp visits	(2) Hospital days	(3) MH visits	(4) Overnight MH
OLS	-0.225	0.001	0.007	0.008
	(0.175)	(0.067)	(0.006)	(0.025)
IV	-0.767	-0.203	-0.074*	-0.048
	(1.470)	(0.965)	(0.043)	(0.185)
Untreated complier mean	5.46	1.42	.199	.304
Sample mean	4.89	1.38	.189	.182
AR p-value	0.602	0.833	0.081	0.795
First stage F	274	274	274	274
N	96173	96173	96173	96173
N indiv.	20752	20752	20752	20752

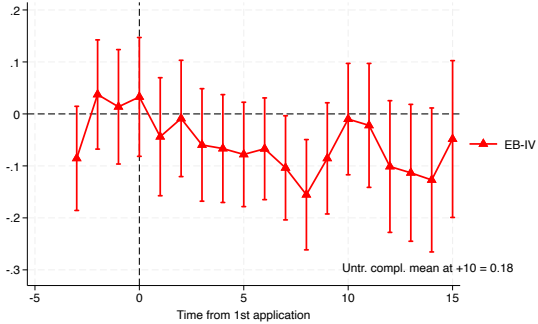
Notes: averages over year 8-15 after application.

Effects on healthcare use

N of hospital visits

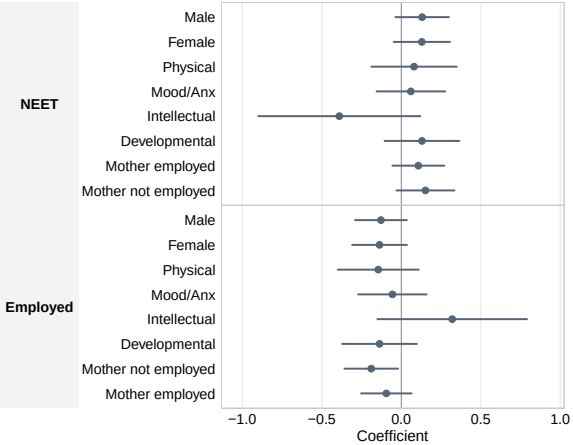


Any MH visit



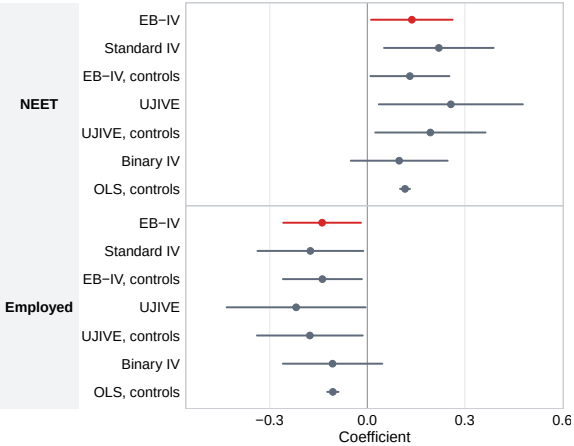
Back

Heterogeneity & Other results



- Increases mostly in non-health related benefits, not DI Different benefits
- Other results: Long-run crime etc.
- Compliers have less severe disabilities, more depression/anxiety/adhd

Robustness



Different endogeneous variable

What did we learn?

- Short run:
  - For most individuals, the counterfactual state was education, but without the YRA
  - Some reduce working, some switch from the adult DI program
  - Ambiguous effects on health, no evidence on harmful consumption
- Long run:
  - Employment decreases
  - Decrease in mental health visits, not in other outcomes

What did we learn?

- Thinking back to the conceptual framework, what was learned?
  - + Increased attendance in schooling ***Yes***
  - + Improved health – reduced stress and increased investments ***Possibly***
  - - Decrease in work experience acquired during the benefit ***Yes***
  - - Information and beliefs about social insurance ***No increase in DI, which is the closest benefit***
  - - Increased consumption of harmful goods ***Not observed***

How does this square up with previous evidence from other programs?

- Successful CCTs in LICs (eg. (Attanasio et al., 2012; Parker and Todd, 2017; Barrera-Osorio et al., 2019))
- In HICs, little evidence how temporary transfers affect long-run outcomes
  - EMA in the UK: null effects (Britton et al., 2025)
  - SSI in the US: reduces activity, prevents crime (Deshpande, 2016; Deshpande and Mueller-Smith, 2022)
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  - Papers with null/negative effects of temporary transfers (Price and Song, 2025; Markussen, 2012)
- ***This paper is the first to examine the potential of a health based benefit to encourage schooling and improve long-run outcomes***
- Cash transfers can have beneficial effects on health (Gelber et al., 2023; Huynh and Chan, 2024; Hämäläinen et al., 2025) or not (Miller et al., 2024; Silver and Zhang, 2022; Päällysaho, 2026) or negative (Black et al., 2024)
- ***Find positive effects, but only in the long run, only on mental health and not on most severe outcomes***

A toy model of human capital accumulation

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  - It would be even better if we could explain the results AND be consistent with previous studies on effects of cash benefits

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  - It would be even better if we could explain the results AND be consistent with previous studies on effects of cash benefits
- Staples of labor economics: education and work experience (Mincer, 1958)
  - Also where we saw effects in the short run
- A very simple model borrowing from (Diamond and Sheshinski, 1995; Keane and Wolpin, 1997) and more modern models of Progresa (eg. (Attanasio et al., 2012))

A model of human capital accumulation: setup

- 2 periods:
  - P1 (adolescence): $d \in$ School (S), employment (E) or inactivity (I)
  - P2 (adulthood): $d \in$ Employment or inactivity
- 3 state variables: educational human capital h, work experience x and health θ
- School accumulates h, employment x.

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- θ acts as a disutility on work and school, assume uniform distribution
- All individuals start having applied to a grant G. If accepted to grant, will receive g and P1 choice will be schooling

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- 3 state variables: educational human capital h, work experience x and health θ
- School accumulates h, employment x.
- Wages are determined by $w(h, x) = w_0 + w_1 \cdot h + w_2 \cdot x + \epsilon$
- θ acts as a disutility on work and school, assume uniform distribution
- All individuals start having applied to a grant G. If accepted to grant, will receive g and P1 choice will be schooling
- Denote employment in P2 as $Y = 1\{d_2 = E\}$
- We are looking to explain why for some individuals we have a negative LATE, that is $(Y|G = 1, \theta = \theta_i) - (Y|G = 0, \theta = \theta_i) < 0$ for some θ_i.

2 types of transitions

- We assume that $d_1 = S$ if $G = 1$, so rules out 6 transitions
- Lets assume $w_2 > w_1$
- Now there can exist 2 groups of individuals who change their choices in P1:
  1. A group that transitions from $E \to S$ in P1
  2. A group that transitions from $I \to S$

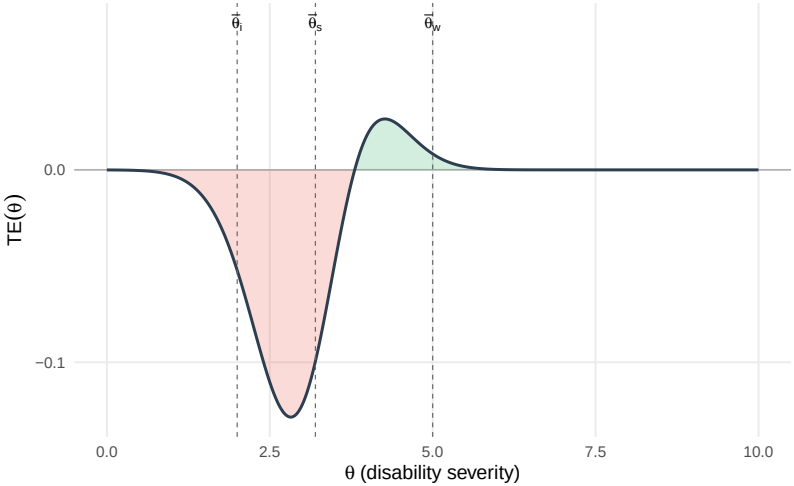
		Potential P1 choice under $D = 0$		
		School	**Work**	**Inactivity**
Potential P1 choice under $D = 1$	**School**	$s \to s$ ruled out school dominated	**Possible switch** $w \to s$	**Possible switch** $i \to s$
	Work		Ruled out:	
	Inactivity	under $D = 1$, accepted applicants are forced into school.		

2 types of transitions

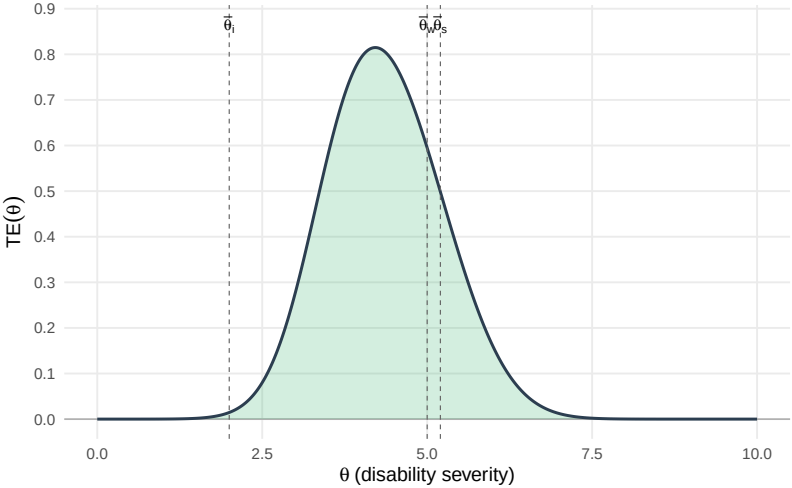
- This group will lose out in the second period (in terms of employment, not total welfare)
- The group that transitions from $I \to E$ in P1 will have weakly higher $P(d_2 = E)$

		Potential P1 choice under $D = 0$		
		School	**Work**	**Inactivity**
Potential P1 choice under $D = 1$	**School**	$s \to s$ ruled out school dominated	**Possible switch** $w \to s$	**Possible switch** $i \to s$
	Work		Ruled out:	
	Inactivity	under $D = 1$, accepted applicants are forced into school.		

Heterogeneous treatment effects



What happens if $w_1 = w_2$?



Further reflections

- Effects of a benefit conditional on something depend on the conditionality
- Probably more room for this if the money itself does not have a high return (school, healthcare almost free-of-charge)
- Could be enriched in different ways and the key conclusions would remain
  - Work and school at the same time
  - Evolution of health capital
  - Variation in w_1 and w_2
  - Is this worthwhile?
- Would it be useful to provide evidence on w_1 and w_2?
  - Previous evidence might not be very applicable here

Conclusion

- I examined the effects of a temporary cash transfer for adolescents with health problems
- Short run: some intended effects
- Long-run: reduced labor market participation
- A simple model of human capital accumulation can explain the results

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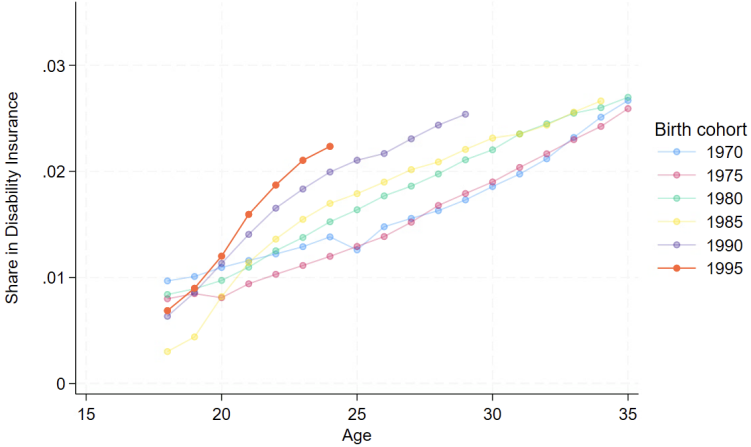
Motivation: Disability insurance increasing in young individuals



Source: own calculations, data from Finland 1987-2022

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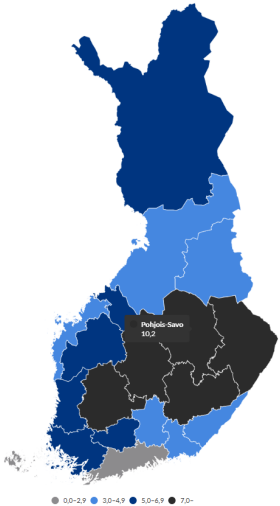
Recipients by area (2023)

Nuoren kuntoutusrahan saajien osuudet (%) maakunnittain
vuonna 2023, 16–19-vuotiaat



1) Northern Savo 10.2%
2) Northern Karelia 8.9%
3) Southern Savo 8.1%
4) Middle Finland 7.9%
5) Pirkanmaa 7.5%

Source: Kela (https://tietotarjotin.fi/tietopaketti/543229/tietopaketti-nuoren-kuntoutusraha) Back

Choice of the endogenous variable

- Can choose from multiple endogenous variables
  - First application accepted
  - Ever accepted
  - Amount of benefits
  - Currently in YRA
- With later outcomes, exclusion restriction more questionable

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	(1)	(2)	(3)
	First application accep.	Ever in YRA	Total YRA Benefits
CEAR	0.673***	0.464***	12272.8***
	(0.0453)	(0.0420)	(1574.1)
N	24855	24855	24855
F-Statistic	220.4	121.8	60.79

Standard errors in parentheses

* $p<0.1$, ** $p<0.05$, *** $p<0.01$

Formula of the instrument

$$CEAR_{ij} = \frac{1}{N_j - 1} \times \left(\sum_{k=0}^{N_j} (Accepted^*_{jk}) - Accepted^*_{it} \right) \tag{1}$$

where N_j is the number of cases handled by examiner j, and $Accepted*$ is the residual of the acceptance decisions, that is the share of the acceptance decision that cannot be explained by the residualization variables.

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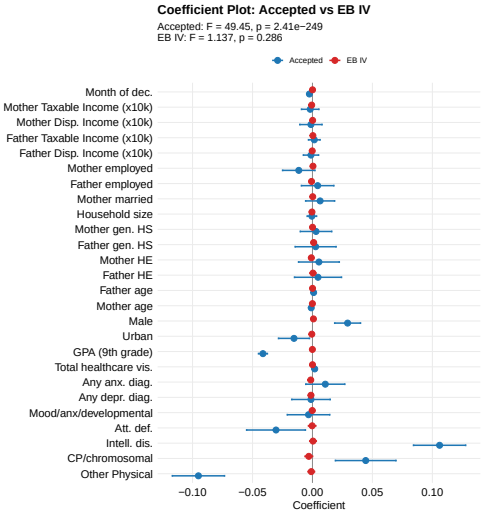
Balance III



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Split-sample IV

	Coef.	N	F
Female [Male]	0.433***	11309	89.36
	(0.0458)		
Male [Female]	0.363***	13531	92.67
	(0.0377)		
Int./chrom. dis. [Others]	0.106***	21318	31.50
	(0.0188)		
Mood/anx [Others]	0.162***	19404	55.78
	(0.0216)		
Dev. dis. [Others]	0.300***	17046	70.30
	(0.0358)		
Physical [Others]	0.223***	13723	38.48
	(0.0360)		

Standard errors in parentheses

* $p<0.1$, ** $p<0.05$, *** $p<0.01$

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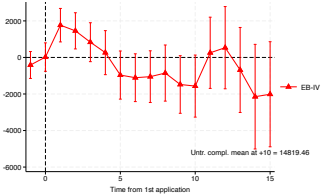
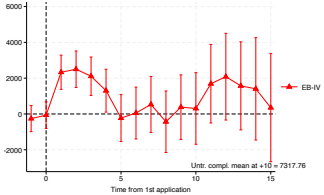
Effects on welfare use

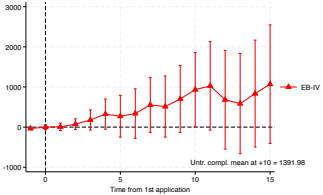
Disposable income



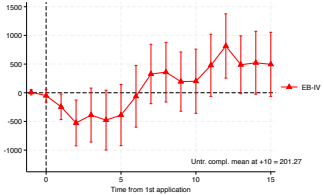
Total transfers



Unemployment benefits



Social assistance



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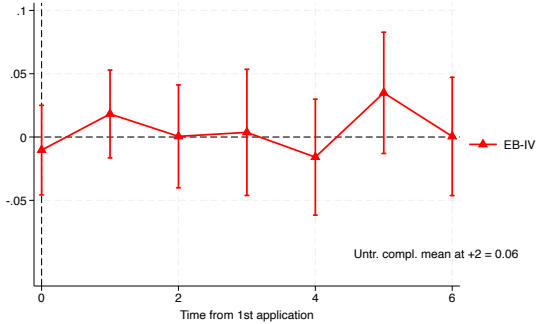
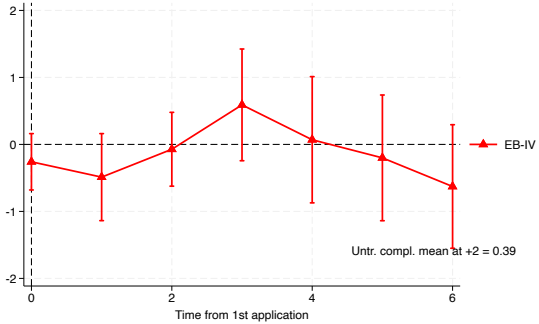
Risky behaviours: Crime

Any criminal conviction



N of crime reports



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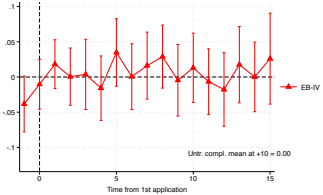
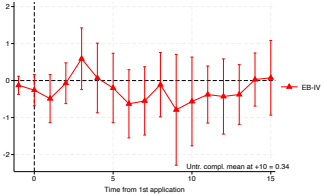
Effects on crime etc

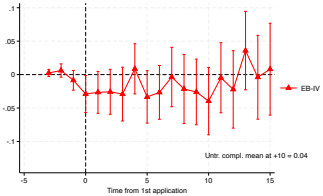
Any conviction



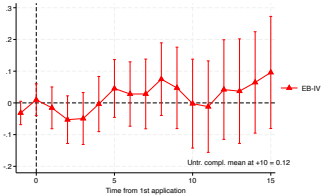
N of crime reports



Any addiction diagnosis



Debt enforcement



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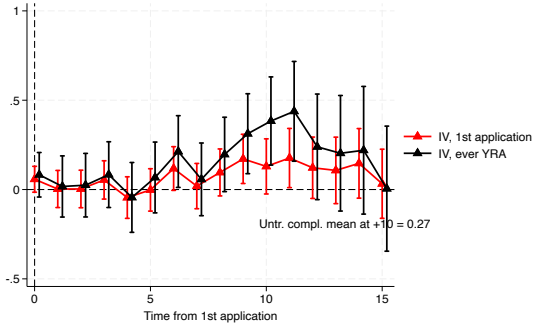
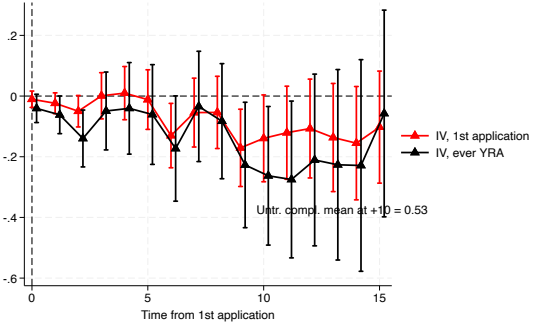
Ever YRA

NEET



Employed (full time)



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